

IN CASE OF TOO MUCH

Bathroom Emergency Guide

In case of too much

You are in a room with a door, water, and one next action.
That is enough to begin.

112 life or lasting harm	116 117 urgent medical help	116 123 a human voice	110 police / active threat
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A small decision system for panic, pain, responsibility, danger, overload, bad smells, missing places, and the occasional collapse of civilization.

The jokes are optional. The red flags are not.

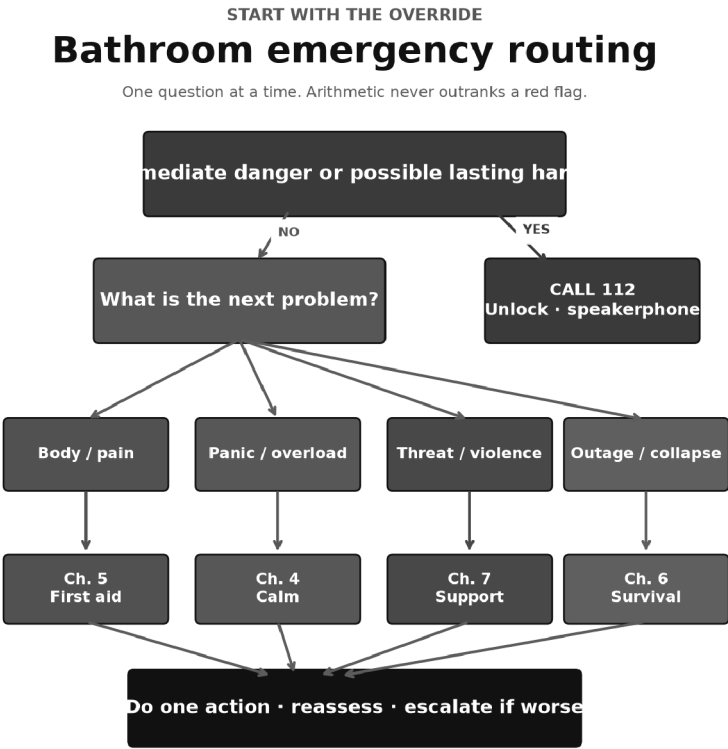
This guide supports decisions; it does not diagnose, replace first-aid training, or overrule emergency dispatchers. If life may be in danger or lasting harm cannot be excluded, call **112 before reading further**.

The override

Could someone die, lose consciousness, stop breathing normally, bleed heavily, suffer lasting harm, or be in immediate danger?

- **Yes / maybe / cannot tell:** call **112**. Unlock the door if safe, use speakerphone, and follow the dispatcher.
- **No:** choose the closest route below.
- **Active violence or a crime requiring police:** get to safety, then call **110**. Use **112** when medical rescue is also needed.

You do not have to diagnose the situation before asking for help. “I am not sure, but this looks serious” is a complete reason to call.



THE ROUTING INVARIANT

Every non-emergency path ends in an action and an escalation rule.

112 life/lasting harm · 116 117 urgent medical · 116 123 human voice

You are not required to diagnose the situation before asking for help.

Red-flag-first emergency decision flowgraph

Four routes

Red flag Call 112. Then do only what the dispatcher asks and what is safe.	Body / pain Go to Ch.5. Start with red flags, then simple first aid.
Panic / overload Go to Ch.4. Orient outward, slow down, choose one action.	Threat / responsibility Go to Ch.2 or Ch.7. Safety first; repair comes after stabilization.
Outage / collapse Go to Ch.6. Verify the hazard, conserve resources, coordinate.	

The seven doors into the guide

DOOR	WHAT IS HAPPENING	FIRST DESTINATION
A	I caused trouble or someone depends on me	Ch.2 Responsibility
B	I feel anxious, panicky, or unreal	Ch.4 Calm
C	I feel pain or physically unwell	Ch.5 First aid
D	I feel endangered	Ch.7 Safety and support
E	Everything is congesting	Ch.4 Calm, then one task
F	There is a bad or unknown smell	Ch.3F Safety check
G	I have no place to go	Ch.7 Practical support

Several doors may be open. Red flags outrank all of them.

Basic theorem 1 — red-flag dominance

Let each red flag be a Boolean value $r_i \in \{0, 1\}$:

$$R = r_1 \vee r_2 \vee \cdots \vee r_n$$

If $R = 1$, the route is **112**. No anxiety score, pain score, formula, or bathroom philosophy may cancel that result. This is a routing rule, not a medical model.

Basic theorem 2 — the one-next-action rule

Acute stress reduces the amount of information people can reliably hold and use. The guide therefore keeps a tiny queue:

$$Q = [\text{one action, one backup, one escalation rule}]$$

Example: **sit down** → **call a friend** → **if chest pain or fainting appears, call 112**. A plan with twelve beautiful steps is decorative. A plan with one executable step is equipment.¹

Reassessment loop

1. **Act:** do the smallest safe action.
2. **Check:** better, same, or worse?
3. **Escalate:** if worse, new red flags appear, or uncertainty remains.
4. **Repeat:** only while the situation remains non-emergent.

This is the guide's only intentional loop. It prevents both frozen inaction and heroic improvisation.

::: {#02-situation-a .chapter} # Situation A — I Caused Trouble

Guilt wants a trial. Emergencies need a sequence.

First split: live problem or aftermath?

A live problem

If a person is injured, unresponsive, not breathing normally, bleeding heavily, giving birth unexpectedly, or at risk of lasting harm:

1. Call **112**.
2. Make the scene safe without creating a second casualty.
3. Follow Ch.5 and the dispatcher.
4. Do not spend the useful minutes deciding whose fault it is.

If violence is active, leave the immediate danger if you safely can. Call **110** for police; call **112** for medical rescue or when immediate danger to life exists.

The aftermath

When nobody is in immediate danger, use the repair sequence:

A = (stop, stabilize, tell, repair, follow up)

- **Stop:** stop the harmful action or process.
- **Stabilize:** prevent further harm.
- **Tell:** inform the affected person or responsible professional plainly.
- **Repair:** replace, compensate, apologize, document, or obtain care.
- **Follow up:** check whether the repair worked.

This is not absolutism algebra. It is simply harder for shame to sabotage five verbs than one enormous moral fog.

Pregnancy, birth, and reproductive decisions

A pregnancy is not a flowchart puzzle and viability statistics do not decide a person's values.

- **Unexpected active birth, heavy bleeding, seizure, fainting, severe pain, severe breathlessness, or serious concern:** call **112**.
- **Urgent but not life-threatening symptoms:** contact maternity care, a gynecological service, or **116 117** outside practice hours.
- **Decision or conflict about a pregnancy:** use a recognized pregnancy counselling service. A calm, confidential appointment is more useful than a milestone table copied into a bathroom.
- **Medication exposure during pregnancy:** contact a clinician or a specialist service such as Embryotox; do not stop prescribed medication solely because of this guide.

If a baby has just arrived unexpectedly, keep parent and baby warm, do not pull on the cord, call **112**, and follow the dispatcher's instructions. Clever improvisation has been officially cancelled.

Child, dependent adult, animal, or other responsibility

Ask four concrete questions:

QUESTION	IF YES	IF NO
Is anyone unsafe now?	112 / 110 / veterinary emergency	Continue
Is essential care missing today?	Arrange food, medication, supervision, shelter	Continue
Is this beyond one person's capacity?	Call family, care service, youth office, vet, or crisis support	Continue
Is exhaustion making aggression feel possible?	Create distance safely and get immediate support	Make a follow-up plan

Caregiver strain is not a character defect. It is a load problem. Load problems need relief, rotation, and services—not more private heroism.²

Harmed someone

- **Accident:** stabilize, get medical help, preserve facts, and report what happened honestly.
- **Anger or loss of control:** create distance, remove access to weapons or dangerous objects if safe, and contact crisis or professional support.
- **Self-defence or coercion:** get safe and obtain legal advice; do not rely on a paragraph summary of German criminal law.
- **Thoughts of harming yourself or another person:** do not stay alone with the means to act. Call **112** for acute danger, or **116 123 / 116 117** for immediate support when there is no acute danger.

A silicon-based life form escaped

Fine. The guide accepts one science-fiction branch.

1. Disconnect or isolate the affected system if authorized and safe.
2. Do not wipe it, “test one more thing,” or publish credentials.
3. Preserve logs and timestamps.
4. Notify the system owner or incident-response contact.
5. If critical infrastructure or a serious cyber incident is involved, consult BSI guidance.

Contain first. Post-mortem later. The server does not need your guilt, only its logs.

Accountability theorem

Self-punishment and repair are not the same variable:

remorse≠repair

Useful accountability increases truth, safety, and restitution. If an action only increases your suffering while changing nothing for the affected party, it may be punishment—but it is not yet repair.

::: {#03-situations-b-g .chapter} # Situations B–G — Pick the Closest Door

B — I feel anxious

First, check the override: new chest pressure, severe breathlessness, fainting, one-sided weakness, confusion, a seizure, or a feeling that this may be a medical emergency means **112**, not “probably anxiety.”

If no red flag is present:

1. Put both feet on the floor or sit safely.
2. Name five things you can see and three sounds you can hear.
3. Let the exhale be gentle and slightly longer than the inhale.
4. Text or call one person: “I am overloaded. Stay with me for ten minutes.”
5. Continue with Ch.4.

The **GAD-7** asks about symptoms over the last **two weeks**. It can support a conversation with a clinician; it is not an acute panic triage score and it cannot rule out a physical emergency.³

C — I feel pain

Pain scores describe experience; they do not measure tissue damage, blood loss, or urgency.

You may use a 0–10 number to communicate change:

$$\Delta P = P_{\text{now}} - P_{\text{earlier}}$$

That difference is useful for a conversation. It is not a diagnosis. Sudden severe pain, chest pain, severe abdominal pain, new neurological symptoms, major injury, pregnancy-related red flags, or pain with collapse routes to **112**. Otherwise continue with Ch.5 or **116 117**.

D — I feel endangered

Danger is present now

- Move toward an exit, other people, or a lockable safe place—whichever reduces danger rather than trapping you.
- Keep noise and screen light low if discovery creates risk.
- Call **110** for police. Call **112** for medical rescue or immediate danger to life.
- Do not confront the person to obtain a cleaner narrative.

The danger is a memory, message, or expected encounter

Save messages and timestamps, tell one trusted person, plan transport and a place to stay, and contact a specialist service. The Germany-wide Violence against Women Helpline is **116 016**, anonymous, free, multilingual, and available around the clock.⁴

E — Things are congesting

Overload often feels like every task became urgent at once. They did not.

Write three lines:

1. **Must prevent harm today**
2. **Must happen soon**
3. **Can survive being ugly**

Now choose one action under five minutes. The rest goes on paper, not in working memory.

A useful capacity model is:

$$L = I + E + S$$

where *I* is intrinsic task difficulty, *E* is avoidable clutter, and *S* is stress load. You may not be able to reduce *I* immediately, but closing tabs, silencing notifications, sitting down, or asking someone else to hold a task reduces *E* or *S*.⁵

F — Bad smell

Do **not** light a match to diagnose or “neutralize” an unknown smell.

- **Gas, solvent, burning plastic, chlorine, rotten-egg smell, dizziness, eye irritation, or headache:** do not operate electrical switches or create flames. Leave for fresh air, warn others, and call **112** from outside if poisoning or fire may be involved. Contact the utility or fire service as appropriate.
- **Sewage smell:** ventilate if safe, run water into rarely used drains, and report a persistent problem to building management or a plumber.
- **Ordinary bathroom smell:** ventilation, cleaning, and a closed toilet lid are acceptable technologies. Fire remains unnecessary.

Never mix household cleaners, especially chlorine bleach with acids or ammonia.

G — No place to go

“No place” may mean no safe home, no room in a group, or no internal place where thoughts stop shouting. The immediate question is practical:

- Where can you be safe for the next hour?
- Who can know where you are?
- What essential item must stay with you: medication, phone, charger, ID, keys?
- Which service can help with tonight, not your entire biography?

For acute danger use **112 / 110**. For a psychological crisis without acute danger use **116 123**, **116 117**, a local crisis service, or a hospital psychiatric emergency department. For housing insecurity, contact the local municipal emergency housing or social service; Ch.7 has a call script.

Consolidated route

SITUATION	DO NOW	ESCALATE WHEN
Anxiety	Orient outward; contact one person	Red flag, self-harm risk, or worsening
Pain	Check red flags; simple first aid	Sudden/severe/new neurological or chest symptoms
Danger	Move to safety; call police/rescue	Threat is current or injury exists
Congestion	Externalize; choose one action	Basic care or safety is failing
Smell	No flame; leave unknown fumes	Fire, gas, chemical exposure, symptoms
No place	Secure one hour; contact a service	Exposure, violence, medical or self-harm risk

Before calming: one safety check

If symptoms are new, severe, physically alarming, or unlike your usual anxiety, use Ch.5 and call **112** when life or lasting harm may be at risk. Calming is not a test you must pass before deserving medical help.

The 90-second landing

1. Make contact

Sit or lean somewhere stable. Feel the floor, wall, or sink supporting weight. Unclench the jaw. Drop the shoulders one centimetre—not spiritually, literally.

2. Orient outward

Find:

- 5 visible objects
- 4 contact points with your body
- 3 sounds
- 2 colours
- 1 next action

Grounding is an attention task. It need not produce enlightenment; it only has to give the alarm system something ordinary to process.⁶

3. Breathe comfortably

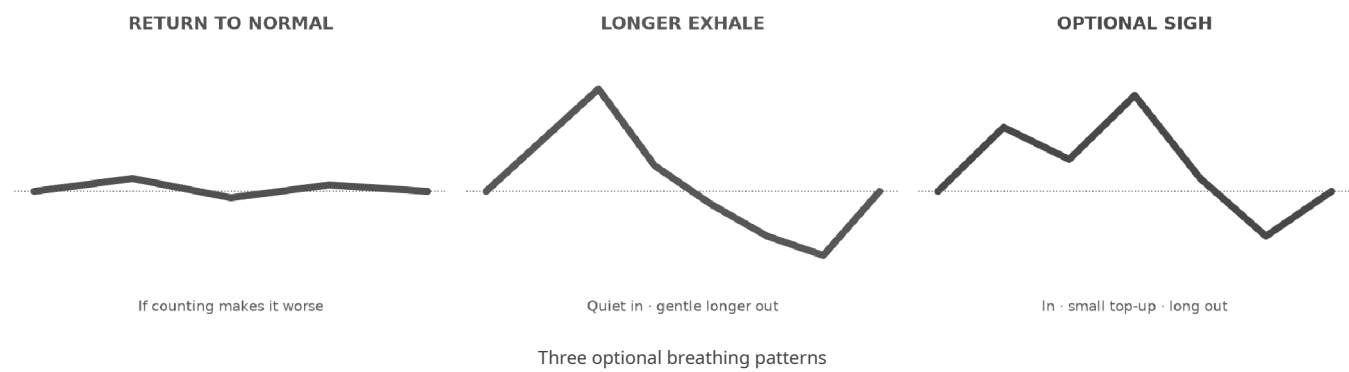
Try a quiet inhale and a slightly longer, unforced exhale. No giant gulps, no competition, no heroic breath-holding. Stop deliberate breathing if it makes you dizzy, tingly, or more frightened and return to normal breathing.

If inhale time is t_i and exhale time is t_e , one cycle is

$$T = t_i + t_e, \quad f = \frac{60}{T}$$

where f is breaths per minute. This formula describes pacing; it does not prescribe one correct rate. Comfort and absence of dizziness outrank the number.

Breathing is a menu, not an exam



A small menu, not a breathing Olympics

Longer exhale

Inhale gently for about 3 counts and exhale for about 4 or 5. Repeat only while comfortable.

Physiological sigh

One inhale, a small second top-up inhale, then a long gentle exhale. One to three repetitions may feel useful. A 2023 randomized study found that five minutes of daily cyclic sighing over a month improved mood and reduced respiratory rate; it did **not** prove that a single sigh treats every acute panic episode.⁷

Box breathing

Equal inhale, pause, exhale, pause can provide structure. Skip the pauses if breath-holding feels bad or if a clinician has advised against it.

The best technique is the one you can do without turning breathing into another exam.

The bathroom control panel

Check the boring variables:

VARIABLE	TINY CORRECTION
Temperature	loosen clothing, cool or warm the room
Water	take a normal drink if thirsty and swallowing is safe
Blood sugar / food	eat something familiar if you have not eaten and can do so safely
Noise / light	reduce one source
Medication	take only as prescribed; do not improvise doses
Contact	tell one person what is happening
Information	stop searching symptoms for ten minutes

A conceptual calming model

Arousal does not have a universal exponential “cortisol decay curve.” Stress chemistry, attention, context, and safety interact. A more honest model is a stepwise process:

$$A_{k+1} = A_k - \delta_k + \varepsilon_k$$

Here δ_k is the small reduction from a useful action and ε_k is whatever the world adds back. This is a teaching model, not human physiology. The theorem is modest: several small reductions can matter even when calm does not arrive in one cinematic wave.

Unhook from the thought

Try the WHO phrasing:

“I notice the thought that ...”

Not “the thought is false,” not “I must defeat it.” Just place one grammatical layer between you and the sentence. Then ask: what action fits the person I want to be for the next five minutes?

Leaving the bathroom

Use a three-line exit plan:

1. **Destination:** “I am going to the sofa / outside / to another person.”
2. **Sentence:** “I got overloaded and needed a minute.”
3. **Backup:** “If I spike again, I will step out and call X.”

Open the door only when the next environment is safe. If the bathroom is the safe place during violence, use Ch.3D and Ch.7 instead.

When calm is not enough

Seek professional help when episodes recur, avoidance grows, sleep or daily function is deteriorating, substances are becoming the main coping tool, or you are frightened of what you might do. Use **116 117** for urgent non-life-threatening medical help, **116 123** for crisis conversation, and **112** for acute self-harm or other immediate danger.

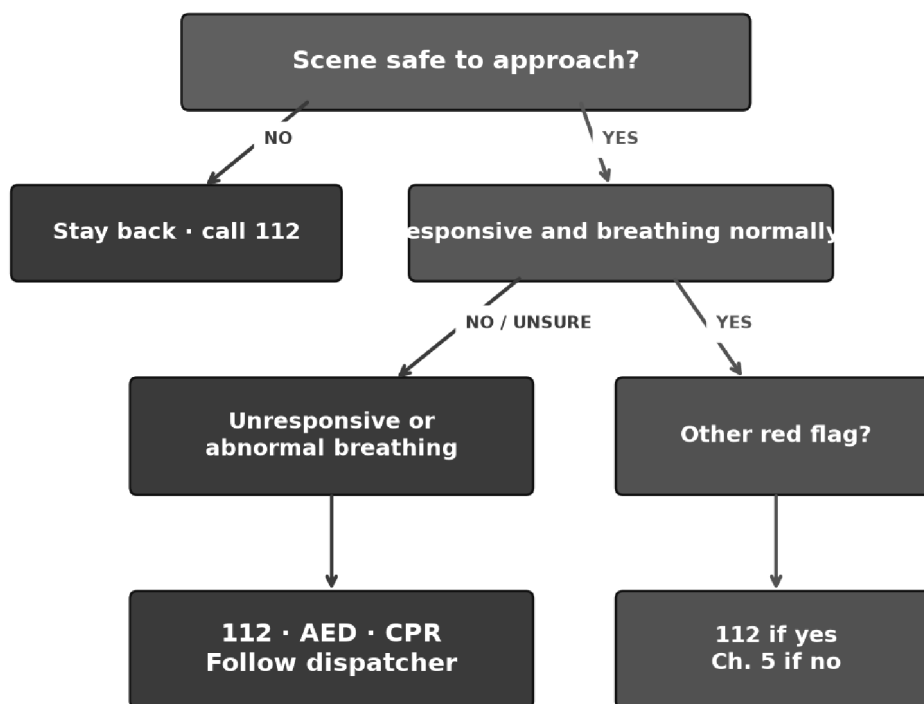
::: {#05-self-ambulance .chapter} # First Aid — You Are the First Link, Not the Whole Ambulance

This chapter is a prompt, not a first-aid qualification. Call **112** early and follow the dispatcher. Put the phone on speaker. Unlock the door if safe.

The first minute

1. **Safety:** do not enter smoke, gas, traffic, electricity, violence, or chemical contamination.
2. **Response:** speak loudly and gently tap the shoulders.
3. **Call:** unresponsive, abnormal breathing, severe bleeding, or another red flag means **112**.
4. **Breathing:** look for normal breathing. Gasping is not normal breathing.
5. **Act:** use the matching section below.

FIRST-AID FIRST MINUTE



Gasping is not normal breathing. Do not delay the call to finish the chart.

First-aid triage overview

Unresponsive and not breathing normally

- Call **112** and send someone for an AED if available.
- Begin chest compressions in the centre of the adult chest: **100–120 per minute, 5–6 cm** deep, allowing full recoil.
- If trained and willing, use **30 compressions to 2 breaths**. If not, keep doing continuous chest compressions.
- Switch rescuers when possible and use the AED as soon as it arrives.

The dispatcher can coach you. Imperfect compressions are better than elegant inaction.⁸

Unresponsive but breathing normally

Call **112**, place the person in the recovery position if no major trauma prevents safe movement, and keep checking breathing. Do not give food or drink. If breathing becomes abnormal, start CPR.

Choking

If the person can cough effectively, encourage coughing. If they cannot speak, breathe, or cough effectively, call **112** and follow the dispatcher's choking instructions. If they become unresponsive, begin CPR. Do not blindly sweep a finger inside the mouth.

Severe bleeding

1. Call **112**.
2. Press firmly and continuously on the wound with a dressing or clean cloth.
3. Keep the person warm and still.
4. If blood soaks through, maintain pressure and add material; do not repeatedly lift the first dressing to inspect your progress.
5. Leave embedded objects in place and press around them.

A dispatcher may give additional instructions, including use of a tourniquet when appropriate. Do not abandon direct pressure to search for perfect gear.⁹

Burns

- Stop the burning process and remove the person from danger.
- Cool the affected area locally with gently running, lukewarm water for about **10 minutes**, stopping if the person becomes cold.
- Remove jewellery or loose clothing near the burn, but not material stuck to skin.
- Cover loosely with a clean non-fluffy dressing.
- Do not use ice, butter, toothpaste, flour, creams, or burst blisters.
- Call **112** for severe or extensive burns, breathing injury, electrical or chemical burns, facial burns with breathing risk, or any serious concern.

German Red Cross public guidance emphasizes local cooling without causing hypothermia.¹⁰

Chemical in eye or on skin

Protect yourself. Remove contaminated clothing if safe and flush immediately with plenty of clean running water, directing runoff away from unaffected skin. For eye exposure, hold lids open and remove contact lenses only if easy. Call **112** for serious symptoms and contact a poison centre for substance-specific advice.

Stroke — FAST

- **F — Face:** one side droops?
- **A — Arms:** one arm weak or drifting?
- **S — Speech:** slurred, strange, or absent?
- **T — Time:** call **112 immediately** and note when the person was last known well.

Do not wait for several signs. One sudden FAST sign is enough to call.

Chest pain or severe breathlessness

Call **112** for strong chest pressure or pain, breathlessness, cold sweat, collapse, pain spreading to arm/jaw/back, or serious uncertainty. Let the person rest in the position that makes breathing easiest. Do not drive yourself.

Anaphylaxis

Sudden breathing difficulty, throat or tongue swelling, collapse, or rapidly progressing symptoms after an allergen is an emergency.

- Use the person's prescribed adrenaline auto-injector immediately according to its instructions.
- Call **112**.

- Keep them lying down unless breathing is easier sitting up; do not let them stand or walk.
- Follow the dispatcher and device instructions for any further dose.

Antihistamines do not replace adrenaline in anaphylaxis.

Poisoning

- Acute collapse, breathing difficulty, seizure, severe symptoms, or possible life-threatening exposure: **112**.
- Otherwise call a German poison information centre; gesund.bund.de maintains the official route and explains when to use 112.
- Keep the package, label, plant, or a photo available.
- Do **not** induce vomiting and do not give a home “antidote” unless a poison specialist instructs you.
- For inhaled fumes, protect yourself and move to fresh air only if this is safe.

Pain without a red flag

Record location, onset, what changed it, and a 0–10 rating if helpful. A pain number is communication, not triage. Contact a practice or **116 117** when the problem is urgent but not life-threatening, especially if pain is new, persistent, worsening, or impairing function.¹¹

While waiting

- Keep the route to the door clear and pets contained.
- Gather medication list, allergies, ID, and the time symptoms began.
- Do not eat or drink if surgery, reduced consciousness, choking, or severe nausea may be relevant.
- Recheck breathing and responsiveness.
- If the condition changes, tell the dispatcher.

Red-flag theorem, again

$$R = 1 \Rightarrow \text{call 112}$$

Vital signs, pain scores, internet searches, and apparent calm can add information. None of them reliably cancel a red flag for a lay reader.

::: {#06-zombie-guide .chapter} # Zombie Guide — Mostly for Non-Zombie Disasters

No confirmed zombie outbreak is known. Power cuts, floods, heat, contaminated water, smoke, and communication failures are real enough. The fictional label stays because a guide is more likely to be read when it contains one zombie.

Verify before optimizing

1. Check official warnings: **NINA**, Cell Broadcast, local radio, municipality, police, fire service, or BBK.
2. Identify the hazard: fire, flood, chemical release, outage, violence, or ordinary rumour wearing tactical trousers.
3. Decide whether authorities say **shelter** or **evacuate**.
4. Tell one other person what you know and where you are.

Do not leave a safe building merely because a group chat has cinematic energy.

Survival mathematics — model, not prophecy

A standard survival function can be written as

$$S(t) = \exp \left(- \int_0^t h(u) du \right)$$

where $h(u)$ is a hazard rate. Without measured hazards this formula predicts nothing. Its practical lesson is simpler: reduce known hazards—smoke exposure, unsafe water, cold, isolation—rather than inventing a percentage for survival.

DISRUPTION PRIORITIES



Priority pyramid for disrupted infrastructure

Water

For household emergency storage, the 2025 BBK guide recommends ideally **2 litres per person per day**, including about 0.5 L for cooking. For n people over d days:

$$W = 2nd \text{ litres}$$

Add pet needs. Heat, illness, pregnancy, breastfeeding, and physical work can increase requirements. Follow local water advisories.

- Store potable water in clean, food-safe containers.
- If authorities issue a boil-water notice, follow its exact instructions.

- Filtering cloudy water through cloth may remove particles; it does not make microbiologically or chemically unsafe water safe.
- Do not use a generic “drops of bleach” recipe: products and concentrations differ.
- Water contaminated by fuel, solvents, pesticides, or radioactive material is not made safe by boiling.¹²

Air, fire, and carbon monoxide

Never run generators, charcoal grills, camping stoves, or open flames inside a home, garage, cellar, tent, or bathroom. Carbon monoxide is colourless and odourless. Move to fresh air and call **112** if poisoning is suspected.

Unknown gas or chemical smell means no matches and no electrical switches. Leave if safe and call from outside.

Food and medication

- Use refrigerated food first while it is still safe.
- Keep fridge and freezer doors closed during outages.
- Discard food when official safety guidance or obvious spoilage says so; smell alone cannot detect every hazard.
- Keep a medication list and a reasonable reserve agreed with the prescriber or pharmacy. Protect medicines requiring refrigeration.
- Do not forage from a four-item list in an emergency guide. Misidentification is a terrible side quest.

Hygiene

Reserve clean water for drinking and essential food preparation. Use separate containers for drinking and washing. Keep hands clean after toilet use and before food handling. If toilets fail, follow municipal sanitation guidance; contain waste away from food and water.

Power and information

- Charge phones and power banks while power exists.
- Use battery radio and official alerts.
- Keep one paper list of contacts, medications, and meeting points.
- Send concise texts; networks often carry text when voice calls fail.
- Preserve battery by lowering brightness and disabling unnecessary radios.

Group coordination

A fully connected group of n people has

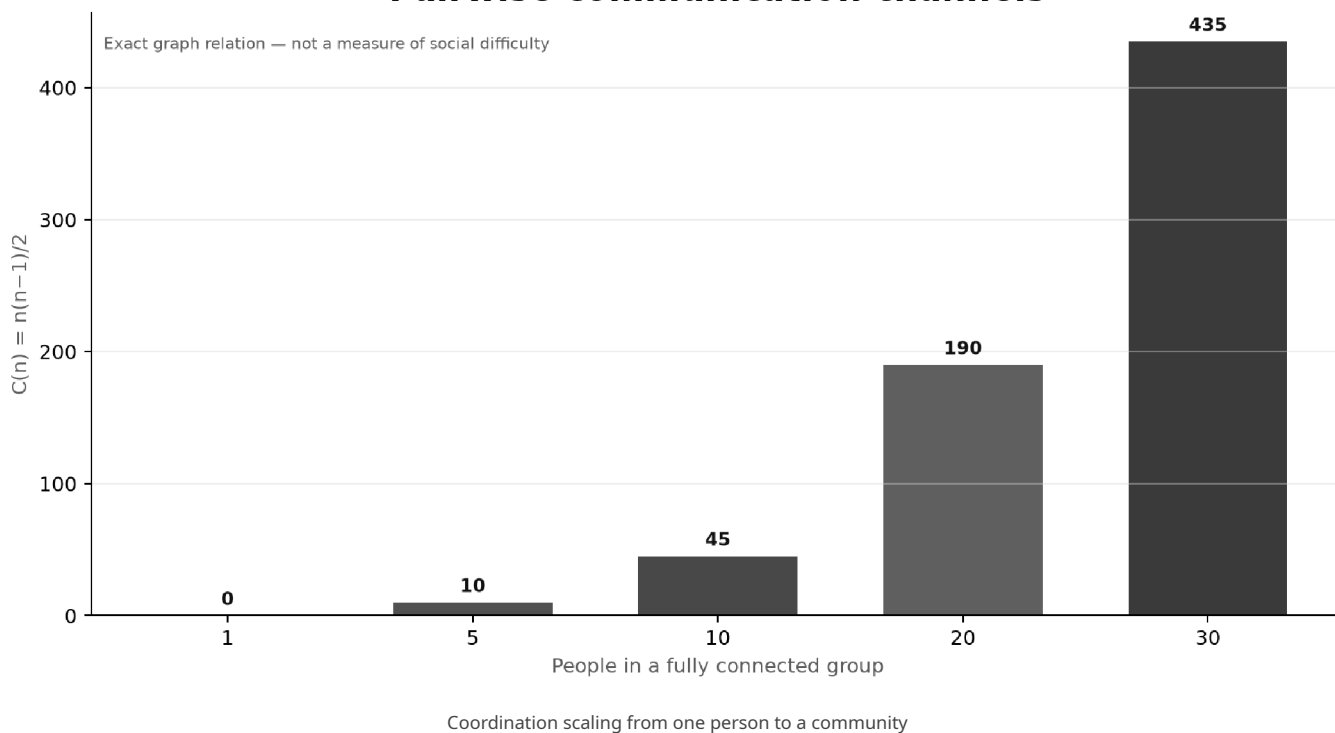
$$C(n) = \frac{n(n-1)}{2}$$

possible pairwise communication channels. At $n = 10$, that is 45 channels; at $n = 30$, 435. The exact formula explains why “everyone tells everyone” collapses quickly.

Use roles and a short briefing:

- safety and first aid
- water and food
- information and communications
- care for children, disabled people, and animals
- logistics and rest rotation

Pairwise communication channels



Commons theorem

Shared resources last longer when the group knows:

1. what the resource is,
2. who may use it,
3. how use is recorded,
4. how rules can be changed,
5. how conflict is resolved.

These echo Elinor Ostrom's empirically grounded design principles for self-governed commons. They are not a magic constitution, but they beat "the loudest person owns the water."¹³

Evacuation pocket list

- phone + power bank
- medication + medication list
- ID, keys, payment method
- water and simple food
- weather layer / sturdy shoes
- small first-aid kit
- glasses, hearing aids, mobility supplies
- infant, disability, and pet supplies
- written destination and contact

Leave weapons, fantasies of looting, and twelve kilograms of philosophical literature unless authorities specifically request an ethics seminar.

::: {#07-professional-support .chapter} # Professional Support — When the Bathroom Is Too Small

Germany quick reference

NEED	CONTACT	USE
Life danger / possible lasting harm / fire	112	Rescue service and fire brigade
Active crime or threat requiring police	110	Police
Urgent medical problem, not life-threatening	116 117	Medical on-call service
Psychological crisis conversation	116 123	TelefonSeelsorge, free and anonymous
Violence against women	116 016	24/7, free, anonymous, multilingual
Children and young people	116 111	Nummer gegen Kummer
Poisoning, no immediate life danger	regional poison centre	Directory via gesund.bund.de
Immediate self- or other-endangerment	112	Rescue service

Numbers and scope were checked against the Federal Ministry of Health portal and the official services on **17 July 2026**.¹⁴

The call script

You do not need the right vocabulary. Use this:

"I am at **[address, floor, door code]**.
 The problem is **[one sentence]**.
 It started **[time]**.
 The person is **[awake/unresponsive]** and **[breathing normally/not]**.
 There is **[danger/no known danger]** at the scene.
 My callback number is **[number]**."

Then answer questions and stay on the line. Do not hang up merely because the first sentence was delivered.

Psychological crisis

Acute danger

If someone may act on suicidal or violent thoughts, cannot stay safe, is severely confused, or has lost contact with reality in a dangerous way, call **112**. Stay nearby only if it is safe. Reduce access to means if you can do so without confrontation or risk.

Asking directly "Are you thinking about killing yourself?" does not plant the idea. It can clarify urgency. A "yes," an unclear answer, or inability to commit to immediate safety deserves real-time professional help.

Urgent support without acute danger

- **116 123** — TelefonSeelsorge
- **116 117** — medical on-call service
- local Sozialpsychiatrischer Dienst
- psychiatric emergency department
- GP or psychotherapist

The public health portal notes that local social psychiatric services are free, accessible, may support relatives, and can sometimes make home visits.¹⁵

Violence and coercion

Do not optimize for a perfect confrontation. Optimize for safety.

- Use **110 / 112** during immediate danger.
- **116 016** supports women affected by violence and people around them.
- Save evidence only when doing so does not increase discovery risk.
- Use a trusted device if the current device may be monitored.
- Arrange a code word, transport, medication, documents, and a safe destination.

A bathroom lock is a delay mechanism, not a complete safety plan.

Medical and reproductive support

- **112**: life danger or possible lasting harm.
- **116 117**: urgent but not life-threatening problems outside practice hours.
- GP / specialist / maternity service for ongoing assessment.
- recognized pregnancy counselling for reproductive decisions.
- Embryotox for evidence-based medication information in pregnancy and breastfeeding.

Children, dependants, and caregiving

Contact pediatric or veterinary emergency services for acute health problems. For care overload, involve family support, Pflegeberatung, youth welfare, school, social services, or respite care before the system relies on one exhausted person.

If you fear you may become aggressive, create safe distance, make sure the dependent person is supervised, and call a crisis or emergency service.

Housing and “no place tonight”

Call the local authority's emergency housing, social service, homeless assistance, or an emergency shelter. Say:

“I have no safe place to sleep tonight. I am currently at **[location]**. I have **[children / disability / medication / pet / safety risk]**. Which service is responsible now, and where can I go?”

If exposure, violence, acute illness, or self-harm risk exists, use **112 / 110**.

Fill these before deployment

LOCAL RESOURCE	NAME / NUMBER
Trusted person	
GP / practice	
Pharmacy	
Local crisis service	
Poison centre	
Safe place / shelter	
Building utility emergency	
Veterinary emergency	

Appendix — The Useful Loose Ends

Evidence labels

The guide now distinguishes three kinds of statement:

LABEL	MEANING	EXAMPLE
Protocol	Action aligned with an authoritative guideline	Call 112 for unresponsive abnormal breathing
Descriptive equation	Exact relation once inputs are known	$C(n) = n(n-1)/2$
Conceptual model	A thinking aid, not a clinical prediction	$A_{k+1} = A_k - \delta_k + \varepsilon_k$
Mnemonic	Memorable approximation	"one action, one backup, one escalation"

The old edition sometimes dressed a metaphor in a lab coat. Version 4 makes the coat show its ID.

Formula and theorem index

1. Red-flag dominance — routing protocol

$$R = \bigvee_i r_i, \quad R = 1 \Rightarrow 112$$

A positive red flag overrides scores and self-reassurance.

2. Tiny action queue — design theorem

$$Q = [\text{action, backup, escalation}]$$

Acute instructions should fit in a small working-memory budget.

3. Pain change — descriptive communication

$$\Delta P = P_{\text{now}} - P_{\text{earlier}}$$

Useful for reporting change; not a measure of injury or urgency.

4. Breath pacing — descriptive equation

$$T = t_i + t_e, \quad f = \frac{60}{T}$$

Describes a chosen pattern. Comfort and safety set the pattern.

5. Household emergency water — planning equation

$$W = 2nd \text{ litres}$$

BBK planning value for n people and d days; individual needs vary.

6. Communication channels — graph theorem

$$C(n) = \frac{n(n-1)}{2}$$

Explains why groups need roles and broadcast channels.

7. Survival function — mathematical model

$$S(t) = \exp \left(- \int_0^t h(u) du \right)$$

Valid mathematics, useless as a personal forecast without measured hazard data.

8. Accountability tuple — procedural mnemonic

$$A = (\text{stop, stabilize, tell, repair, follow up})$$

Turns guilt into observable repair work.

Pocket print card

RED FLAG? 112 · unlock if safe · speakerphone · follow dispatcher Unresponsive · abnormal breathing · severe bleeding · stroke sign · chest pressure · severe breathlessness · seizure · major burn · collapse · acute self/other danger	
BODY Ch.5 · first aid · 116 117 if urgent but non-life-threatening	ALARM Feet · five objects · long gentle exhale · one person
THREAT Exit / safer place · 110 · 116 016 · do not confront	OUTAGE Official warning · water · air/fire · medication · roles

Offline deployment checklist

- Print the monochrome PDF single-sided or duplex.
- Write local contacts in Ch.7.
- Keep the guide near a charged light source.
- Add a simple first-aid poster from an official provider.
- Refresh service numbers and medical guidance at each release.
- Replace the guide after water damage. It is not itself waterproof, despite strong thematic alignment.

Navigation invariant

Every non-emergency route must provide:

1. a next action,
2. a reason to escalate,
3. a named destination.

A dead end in prose is still a dead end.

Version History

4.0.0 — 17 July 2026

Technical realization

- Replaced CDN MathJax and timed browser waiting with native MathML.
- Removed brittle cover-document CSS merging; chapters and cover now share one semantic document.
- Added a Pandoc template with table of contents, mobile navigation, theme toggle, reading progress, and print action.
- Rebuilt print CSS around an enforced A4 single-column invariant.
- Added an independent monochrome layer without copying the full stylesheet.
- Added Chrome and WeasyPrint PDF backends consuming identical HTML.
- Added structural, safety-wording, MathML, image, A4, and page-count validation.
- Fixed chapter assembly so repeated YAML frontmatter does not leak into output.

Orientation and UI

- Replaced route-first diagrams with a portrait, red-flag-first flowgraph.
- Added four explicit destinations and a reassessment loop.
- Reduced decorative cards and gradients in favour of a bathroom-tile, high-contrast utilitarian system.
- Improved mobile contents navigation and print table/figure behaviour.

Content and evidence

- Corrected newborn/medical routing from 110 to 112.
- Removed concentration-free bleach dosing, unsafe foraging advice, match-based smell advice, pain “physiological correlates,” and fictional cortisol decay.
- Reframed GAD-7 and pain scales as communication/screening tools, not triage.
- Rewrote CPR, bleeding, burn, stroke, poisoning, anaphylaxis, crisis, violence, water, outage, pregnancy, and caregiver sections against authoritative sources.
- Marked every formula as protocol, descriptive equation, conceptual model, or mnemonic.
- Expanded current German help numbers and source annotations.

Earlier versions

VERSION	DATE	SUMMARY
3.4	2026-06-10	experimental multi-backend and two-column work
3.2	2026-05-03	formula and scientific-diagram expansion
3.0	2026-05-01	modular content and source chapter
2.0	2026-04-29	pixel assets and themed HTML/PDF
1.0	2026-04-29	initial guide

Version 4 deliberately removes several “scientific-looking” claims introduced in 3.2. More formulas are not automatically more science; better boundaries are.

Sources and Evidence Notes

Checked **17 July 2026**. Official and primary sources are preferred. A link is not evidence by itself; the guide cites what each source actually supports.

Emergency routing and first aid

1. **European Resuscitation Council — Guidelines 2025.** Current European resuscitation framework, including adult basic life support, paediatric and newborn life support, and first aid.
<https://www.erc.edu/science-research/guidelines/guidelines-2025/guidelines-2025-english/>
2. **ERC — Guidelines 2025 for Everyone.** Lay version emphasizing early recognition, emergency activation, high-quality compressions, and AED use.
https://www.erc.edu/media/p5ymaej/gl2025_layperson_book_ipdf-v11-e.pdf
3. **German Red Cross — Finding a person in an emergency.** Scene safety, response and breathing checks, recovery position, emergency call, and CPR.
<https://www.drk.de/hilfe-in-deutschland/erste-hilfe/auffinden-einer-person/>
4. **German Red Cross — Severe bleeding.** Direct pressure, pressure dressing, warmth, monitoring, and 112 for life-threatening bleeding.
<https://www.drk.de/hilfe-in-deutschland/erste-hilfe/blutungen-und-blutstillung/blutungen/>
5. **German Red Cross — Public burn first aid.** Local lukewarm cooling, clean covering, avoidance of home remedies, and escalation for severe burns.
<https://www.drk.de/hilfe-in-deutschland/erste-hilfe/erste-hilfe-handgriffe-fuer-den-sommer/>
6. **German Red Cross — Chemical burns.** Self-protection, removal of contaminated clothing, and immediate irrigation.
<https://www.drk.de/hilfe-in-deutschland/erste-hilfe/veraetzungen/>
7. **gesund.bund.de — Emergency numbers.** Official German distinctions among 112, 116 117, poison centres, and crisis lines.
<https://gesund.bund.de/notfallnummern>

Psychological crisis and calming

8. **World Health Organization — Doing What Matters in Times of Stress** (2020). Evidence-informed, field-tested grounding, unhooking, values, and small-action exercises.
<https://www.who.int/publications/i/item/9789240003927>
9. **gesund.bund.de — Managing psychological crises.** Safety escalation, professional help, social contact, and caution that inward-focused relaxation may intensify some acute crises.
<https://gesund.bund.de/mit-psychischen-krisen-umgehen>
10. **Balban MY et al.** "Brief structured respiration practices enhance mood and reduce physiological arousal." *Cell Reports Medicine* 4 (2023), 100895. The trial studied five minutes of daily practice for 28 days; the guide avoids converting it into a universal one-breath emergency claim.
<https://doi.org/10.1016/j.xcrm.2022.100895>
11. **Spitzer RL et al.** "A Brief Measure for Assessing Generalized Anxiety Disorder: The GAD-7." *Archives of Internal Medicine* 166 (2006): 1092–1097. Supports a two-week symptom screen, not acute medical triage.
<https://doi.org/10.1001/archinte.166.10.1092>

12. **Hjermstad MJ et al.** "Studies comparing Numerical Rating Scales, Verbal Rating Scales, and Visual Analogue Scales for assessment of pain intensity in adults." *Journal of Pain and Symptom Management* 41 (2011): 1073–1093. Supports pain intensity as patient report; not a stand-alone urgency score.
<https://pubmed.ncbi.nlm.nih.gov/21110961/>
13. **Cowan N.** "The magical number 4 in short-term memory." *Behavioral and Brain Sciences* 24 (2001): 87–114. Used as a design reason for short instructions, not a fixed capacity diagnosis.
<https://doi.org/10.1017/S0140525X01003922>

German crisis and support services

14. **TelefonSeelsorge.** Official free and anonymous crisis conversation: 116 123 and additional channels.
<https://www.telefonseelsorge.de/>
15. **Violence against Women Helpline.** Official 116 016 service; free, anonymous, multilingual, 24/7.
<https://www.hilfetelefon.de/>
16. **116117.de.** Official medical on-call service for urgent, non-life-threatening problems.
<https://www.116117.de/>
17. **gesund.bund.de — Social psychiatric service.** Public low-threshold support for people in crisis and their social environment.
<https://gesund.bund.de/sozialpsychiatrischer-dienst>
18. **gesund.bund.de — Caregiver overload.** Warning signs, relief, counselling, and acute-crisis options.
<https://gesund.bund.de/belastungen-pflegende-angehoerige>
19. **Embryotox.** Evidence-based medication information and specialist counselling for pregnancy and breastfeeding.
<https://www.embryotox.de/>
20. **familienplanung.de.** Federal public information and recognized pregnancy counselling search.
<https://www.familienplanung.de/beratung/beratungsstellensuche/>

Preparedness, water, and groups

21. **Bundesamt für Bevölkerungsschutz und Katastrophenhilfe.** *Vorsorgen für Krisen und Katastrophen* (2025). Current German household preparedness, 2 L/person/day water planning value, individual needs, warnings, evacuation supplies, hygiene, and outage advice.
https://www.bbk.bund.de/SharedDocs/Downloads/DE/Mediathek/Publikationen/Buergerinformationen/Ratgeber/BBK-Vorsorgen-fuer-Krisen-und-Katastrophen.pdf?__blob=publicationFile&v=44
22. **WHO — Technical Notes on Drinking-Water, Sanitation and Hygiene in Emergencies.** Treatment guidance and the important limits of household methods.
<https://www.who.int/publications/m/item/technical-notes-on-drinking-water-sanitation-and-hygiene-in-emergencies>
23. **Elinor Ostrom.** "Beyond Markets and States: Polycentric Governance of Complex Economic Systems," Nobel Prize Lecture (2009). Empirical basis for commons-governance principles.
<https://www.nobelprize.org/prizes/economic-sciences/2009/ostrom/lecture/>
24. **Inter-Agency Standing Committee.** *Guidelines on Mental Health and Psychosocial Support in Emergency Settings.* Layered support from basic safety and services through specialized care.

<https://interagencystandingcommittee.org/iasc-task-force-mental-health-and-psychosocial-support-emergency-settings/iasc-guidelines-mental-health-and-psychosocial-support-emergency-settings-2007>

Cyber incident branch

25. **Federal Office for Information Security (BSI).** Citizen and organizational incident information and reporting routes.
<https://www.bsi.bund.de/>
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Editorial policy

- Do not infer acute medical urgency from a GAD-7 or pain score.
 - Do not print drug or chemical dosing without product- and concentration-specific authority.
 - Distinguish measured clinical relationships from explanatory models.
 - Prefer a short action plus escalation rule to an impressive but unusable lecture.
 - Recheck service numbers and guideline versions at every release.
-

1. World Health Organization, *Doing What Matters in Times of Stress* (2020):
<https://www.who.int/publications/i/item/9789240003927>↵
2. gesund.bund.de, “Überlastung bei pflegenden Angehörigen” (updated 2025), including crisis and relief options: <https://gesund.bund.de/belastungen-pflegende-angehoerige> ::↵
3. Spitzer RL et al., “A Brief Measure for Assessing Generalized Anxiety Disorder: The GAD-7,” *Archives of Internal Medicine* 166 (2006): 1092–1097.
<https://doi.org/10.1001/archinte.166.10.1092>↵
4. Federal Violence against Women Helpline, official service information:
<https://www.hilfetelefon.de/>↵
5. Cowan N, “The magical number 4 in short-term memory,” *Behavioral and Brain Sciences* 24 (2001): 87–114. The estimate concerns controlled laboratory tasks and is a design clue, not a fixed personal limit. <https://doi.org/10.1017/S0140525X01003922> ::↵
6. World Health Organization, *Doing What Matters in Times of Stress* (2020):
<https://www.who.int/publications/i/item/9789240003927>↵
7. Balban MY et al., “Brief structured respiration practices enhance mood and reduce physiological arousal,” *Cell Reports Medicine* 4 (2023), 100895.
<https://doi.org/10.1016/j.xcrm.2022.100895> ::↵
8. European Resuscitation Council, *Guidelines 2025* and layperson guidance:
<https://www.erc.edu/science-research/guidelines/guidelines-2025/guidelines-2025-english/>↵
9. German Red Cross, “Starke Blutungen”: <https://www.drk.de/hilfe-in-deutschland/erste-hilfe/blutungen-und-blutstillung/blutungen/>↵
10. German Red Cross, public burn first-aid guidance: <https://www.drk.de/hilfe-in-deutschland/erste-hilfe/erste-hilfe-handgriffe-fuer-den-sommer/>↵
11. Federal Ministry of Health portal, “Nummern für den Notfall” (updated 2025):
<https://gesund.bund.de/notfallnummern> ::↵
12. Bundesamt für Bevölkerungsschutz und Katastrophenhilfe, *Vorsorgen für Krisen und Katastrophen* (2025), especially the water and individual-needs checklists:
https://www.bbk.bund.de/SharedDocs/Downloads/DE/Mediathek/Publikationen/Buergerinformationen/Ratgeber/BBK-Vorsorgen-fuer-Krisen-und-Katastrophen.pdf?__blob=publicationFile&v=44↵
13. Elinor Ostrom, “Beyond Markets and States: Polycentric Governance of Complex Economic Systems,” Nobel Prize Lecture (2009): <https://www.nobelprize.org/prizes/economic-sciences/2009/ostrom/lecture/> ::↵
14. gesund.bund.de, “Nummern für den Notfall”: <https://gesund.bund.de/notfallnummern> ; TelefonSeelsorge: <https://www.telefonseelsorge.de/> ; Violence against Women Helpline: <https://www.hilfetelefon.de/>↵
15. gesund.bund.de, “Sozialpsychiatrischer Dienst – Hilfe bei psychischen Krisen” (2025):
<https://gesund.bund.de/sozialpsychiatrischer-dienst> ::↵